



CN 4063



H-2021-0768



SURGICAL DISCHARGE SUMMARY

Patient Name	: MS SATVIKI PATIDAR	Encounter No	: 902604240000
Address	: PARAS PATIDAR ALWASA - 453111 MADHYA PRADESH, INDIA	UHID	: 9000701818
Age / Gender	: 7 Y / FEMALE	Ward	: PVT-B / 1002 / A
Phone	: 8826533558	Date Of Admission	: 24/04/2026 03:33:45
BillType	: ADITYA BIRLA HEALTH INSURANCE CO. LIMITED	Date Of Discharge	: 18/05/2026 12:49:10

DEPARTMENT OF NEUROSURGERY

Dr. Roopesh Kumar V R, Director and Senior Consultant
 Dr. Vijay Sundar, Senior Consultant
 Dr. Ajinkya, Consultant
 Dr. Vigneshwar, Consultant
 Dr. Kaviyaran, Consultant

NEURO ANESTHETIST

Dr. Arulvelan- Senior Consultant
 Dr. Ayshwarya- Consultant
 Dr. Nivaedha- Associate Consultant

DIAGNOSIS: LEFT THALAMIC GLIOMA WITH OBSTRUCTIVE HYDROCEPHALUS

HPE - PEDIATRIC-TYPE DIFFUSE INFILTRATING GLIOMA WITH ASTROGLIAL DIFFERENTIATION, IDH-WILDTYPE AND H3K27-ALTERED NEGATIVE, SHOWING FOCAL EARLY HIGH-GRADE/ANAPLASTIC FEATURES INCLUDING SUBTLE MICROVASCULAR PROLIFERATION, FOCI SUSPICIOUS FOR INCIPIENT NECROSIS, INCREASED P53 EXPRESSION, AND ELEVATED KI-67 LABELING INDEX (6-9% IN HOT-SPOT AREAS) (KNOWN CASE OF DBN1::BRAF FUSION-POSITIVE, MAPK PATHWAY-ALTERED PEDIATRIC-TYPE DIFFUSE LOW-GRADE GLIOMA, CNS WHO 5TH EDITION).

PROCEDURE DONE:

- 1) Right Keen's point Programmable Meithkes Ventriculo-Peritoneal Shunt insertion under navigation guidance undert GA done on 27/4/26
- 2) Left pterional, high frontal craniotomy (combined pterional, subtemporal and interhemispheric trans-callosal approach) and safe maximal resection of left thalamic lesion with neuro navigation and IONM, in supine under ETGA ON 29.04.2026

History of present illness: Ms. Satviki Patidar 7 year old female child, a case of optic pathway hypothalamic glioma with left mesial temporal extension - modified DODGE TYPE 3B, H+, who underwent left pterional craniotomy with microsurgical excision of mesial temporal lesion on 22/4/2025. Histopathology reported as Diffuse low grade glioma with oligoid features, GFAP, S100, Synaptophysin, SOX 10 - Positive, DBN1/BRAF fusion - Positive, 1p/19q - co-deletion - negative. Patient was started on Tovorafenib therapy - started in March 2026. Her father noticed acute worsening in nenurology / ophthalmolgical symtoms post second dose. which was gradually progressed. Now patient presented with complaints of acute weakness of right upper and lower limb, reduced response, difficulty in swallowing which were gradually progressive. Now patient admitted for further management.

Past history:

No known other co-morbidity

VITAL SIGNS

BP: 125/110 mmhg
 PR: 63/min
 RR: 15/min
 SPO2: 99 % on room air
 Temp: Afebrile

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Systemic Examination:

On arrival to ER,
CNS:
GCS - 15/15
B/L PERL
Power - LEFT UL & LL 3/5
CVS: S1/S2 (+)
RS: B/L air entry (+)
Abdomen: Soft, not tender, BS (+)

Course in the Hospital:

7 year old female Satviki Patidar, came with the above mentioned complaints. Immunotherapy started on somewhere else on 24/2/2026 followed by child developed weakness of right upper limb and restriction of eyeball movements and drowsiness, clinically child had features of encephalopathy, hyponatremia correction given, started on NG Feeds. Dr. Anu (Pediatrician) opinion obtained and advise followed. Dr. Priyanka (Endocrinologist) opinion obtained for hyponatremia and advise followed. Dr. Shrivarthan (neurophysician) opinion obtained and advise followed. MRI brain with DTI showed features of increase in tumor size and obstructive hydrocephalus, she developed features of raised ICP and she was electively intubated on 24/04/2026. Dr. Deenadayalan (pediatric Oncologist) opinion obtained and advise followed. CT Brain done on 25/04/2026 showed obstructive hydrocephalus for which she was planned for emergency VP shunt to reduce the ICP. After a written consent she underwent Right VP shunt under GA and was planned for tumor board discussion after shunt.

Procedure notes: 1) Right Keen's point Programmable Meithkes Ventriculo-Peritoneal Shunt insertion under navigation guidance under GA done on 27/4/26

After informed consent was obtained, the patient was taken to the operating room and placed supine under general anesthesia. The head, neck, chest, and right Abdomen were prepared and draped in a sterile fashion.

A right U shaped incision was made at Keen's point. The scalp flap was reflected, and a burr hole was created. The dura was coagulated and opened in a cruciate fashion.

A ventricular catheter was introduced toward the lateral ventricle. The ventricle was cannulated successfully on the first attempt, with free flow of clear cerebrospinal fluid under high pressure, confirming correct intraventricular placement.

The ventricular catheter was secured, and a programmable Meithkes valve was connected. The valve was fixed at the bone level and secured appropriately.

A subcutaneous tunnel was created from the cranial incision to the right chest. A small incision was made over the right abdominal wall, and the peritoneal cavity was accessed carefully. The distal catheter was inserted into the peritoneal space without difficulty, and proper positioning was ensured.

All connections were checked and found to be secure. Hemostasis was achieved. The wounds were closed in layers using absorbable sutures for deeper layers and staples for the skin. Sterile dressings were applied.

Intraoperative Findings

Ventricular catheter placed successfully on first attempt

Clear CSF obtained

CSF under high pressure

No intraoperative complications

Implants

Programmable Meithkes ventriculo-peritoneal shunt system

Post operatively, she was not extubated and shifted to PICU and was managed with IV Fluids, analgesics, antiedema, AED and other supportive measures. Gradually she was weaned off from ventilator on 28/04/2026.

After MDT meeting, she was planned for Craniotomy and safe maximal excision of lesion under navigation.

MRI Brain done on 29/04/2026 navigation protocol showed The left hemispheric lesion shows mild increase in size, measures ~5.9 x 5.9 x 5 cm (AP x TR x CC) [-5.1 x 5.3 x 4.6 cm]. Shows few areas of necrosis within the central part of rosette shaped tumor bulk. The lesion shows few patchy areas of SWI blooming suggestive of hemorrhage. The degree of raised perfusion is less in the current scan within the lesion. Right parietal EVD noted traversing across the body of the lateral ventricle with its tip adjacent to genu of corpus callosum with interval reduction in hydrocephalus. Rest of the findings as mentioned in the previous study. Post MRI child was shifted to OT.

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2) Left pterional, high frontal craniotomy (combined pterional, subtemporal and interhemispheric trans-callosal approach) and safe maximal resection of left thalamic lesion with neuro navigation and IONM, in supine under ETGA ON 29.04.2026

Patient positioned supine neutral in 3 pin and the IONM electrodes placed. The tumour boundaries were defined using neuro-navigation. The surgical site was prepared and draped in a routine manner. We decide to approach the lesion via two incisions, one the previous left pterional incision and another inverted 'L' shaped incision with the long axis along the midline, leaving behind an island of skin flap, to prevent flap necrosis. The incisions were made, first at the high frontal, skin through bone was unremarkable. Using two para-median burr holes, the frontal craniotomy was performed. The underlying SSS along the midline, was dissected away from the bone and a completion craniotomy was performed across the midline, to get better trajectory and access to the lesion. Similarly, the left pterional incision was utilised and the skin through bone as densely replaced by scar tissue. The bone had grown over the previous bioplates and were pulled out with caution and an extended craniotomy was performed.

Interhemispheric, trans-callosal approach: The dura was opened with the base over the SSS. The ipsilateral hemisphere gently retracted to view the corpus callosum, taking care not to injure any draining veins. Callosotomy was performed and the left lateral ventricle was entered into. The tumour was obvious, greyish white, highly vascular with areas of necrosis. The tumour was removed piece meal and with CUSA was debulked. We received good neuro monitoring responses along the lateral and the inferior limit of the lesion, which was decided to be left behind. A safe maximal excision was performed. Thorough antibiotic-saline wash given. Hemostasis was secured. The tumour bed layered with fibrillar and gel foam.

Left pterional, subtemporal: The previous scarred dura was opened, in a curvilinear fashion. It was densely adherent to the underlying parenchyma; from which it was meticulously dissected away. The sylvian fissure identified. We decided to approach sub-temporally, to identify the lesion. The lesion was debulked as piece meal and using CUSA. There were a good number of feeders and prominent vessels around. Here too, we received good neuro monitoring responses along the inferior and medial aspect, possibly the midbrain and this portion was decided to be left behind. Once again, a safe maximal resection was performed. Thorough antibiotic-saline wash given. Hemostasis was secured. The tumour bed layered with fibrillar and gel foam.

The MEPs and IONM signals were good throughout the procedure.

The dura replaced with artificial dura - Duragen. The bone flaps were replaced and secured with bioplates and the remaining defect closed with titanium mesh plate and secured with bioplates and screws. Hemostasis secured and the wounds were closed in layers over a non-charged drain, connected to a 'Y' connector. Skin closed with anchoring Ethilon sutures and staples. Sterile dressing applied.

Post operatively, she was not extubated and shifted to PICU connected to mechanical ventilator and anti edema measures continued she had features of SIADH, serial sodium monitoring and supplementation adjusted. Post op MRI Brain with contrast was done on 30/4/26 showed S/p left frontoparietal craniotomy with combined left pterional and interhemispheric transcalsal approach and safe margin excision of left hemispheric lesion with neuronavigation. Right frontal pneumocephalus noted. Post surgical changes with fluid filled resection in the left side of body of corpus callosum, through the left lateral ventricle and in the left medial gangliothalamic and hypothalamic regions. The cavity is surrounded by SWI blooming and T1 hyperintensities suggestive of hemorrhagic residues. Pneumoventricle noted in frontal horns of bilateral lateral ventricles (L>R). Post left pterional craniotomy changes with surgical resection cavity noted in left medial temporal region surrounded by SWI blooming and T1 hyperintensities suggestive of hemorrhagic residues.

Occupational therapy opinion obtained and advise followed.

Child extubated to room air on 1/05/2026. Dexamethasone tapered and changed to hydrocortisone. Post operatively child had poor sensorium suspected metabolic abnormalities and brain stem involvement neurologist opinion obtained suggested to do EEG and antiepileptics continued as per neurologist Dr. Shrivarthan (Neurophysician), she had persistent encephalopathy constantly improving, responding to commands probably due to primary disease progression.

Under strict aseptic precaution surgical site drain was removed on 3/5/26. Currently she is on NG Feeds 150 ml 3rd hourly with common salt 3 gm in each feed tolerating well, being shift to ward on NG Tube and left PICC line, risk of aspiration, PICC line infection explained to parents.

she developed 1 episode of fever spike 100.4 F on 5/5/2026, CRP sent which was negative, no further fever spikes noted in PICU Stay.

As her condition stable, she was shifted to ward.

SALT review obtained for swallowing assessment and advised to continue RT Feed.

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Gradually she was mobilised out of bed.

Periodic pediatrician review obtained and advise followed.

Patient had hyponatremia (Na - 127) for which she was started on hypertonic saline, periodic sodium monitoring was done and managed appropriately.

She was continued with physiotherapy and other supportive measures.

Follow up MRI Brain plain was done on 11/5/26 showed - Complete resolution of pneumocephalus in right frontal region and in frontal horns of bilateral lateral ventricles. Mild reduction of fluid within the resection cavity in left side of body of corpus callosum. Mid reduction of air pocket and mild increase of fluid within the resection cavity in left medial temporal region. No interval change in size of residual lesion. No change in midline shift. Relative luminal narrowing in M1 segment of left MCA.

Child had high grade fever spikes for which pediatrician review obtained and advised to S. Procalcitonin, CPR, Blood culture and to start Inj.Mero & Vanco.

Gradually her fever spikes were reduced.

Periodic sodium monitoring was done and was managed appropriately.

Repeat EEG was done was showed no epileptiform discharge.

She is tolerating RT feed, self voiding and ambulant with bimanual support.

She is clinically and hemodynamically stable. Hence being discharged and referring to nearby hospital at home town for further management and neuro-rehabilitation to recovery of consciousness.

Condition on discharge:

Vitals stable

CNS

GCS : E1V2M5

Right paucity+

Tone - Hypotonia

Pupil : 3mm sluggish reactive 2mm sluggish reactive

Surgical site wound healthy

On RT feed

Ambulant with bimanual support

Left axillary vein PICC line(Groshong 4Fr single lumen catheter) in situ , inserted on 04/05/2026

Freka NG tube (12Fr) inserted on 15/05/2026

MEDICATIONS:

S NO	MEDICATION	DOSE	FREQUENCY	DURATION
1	SYP. BREVIPIL	15 MG	1-0-1	TO CONTINUE
2	TAB. LACOSET	100 MG	1-0-1	TO CONTINUE
3	TAB. CLOBAZAM	2.5 MG	0-0-1	TO CONTINUE
4	TAB. ADDNA	3 GM - 2 GM - 3 GM		TO CONTINUE
5	TAB. FLUDROCORTISONE	0.1 MG	1-0-1	TO CONTINUE
6	REFRESH EYE DROPS	2 DROPS	EVERY 2ND HOURLY	TO CONTINUE
7	SYP. ZINCONIA	20MG/5ML	1-0-0	FOR 10 DAYS

DISCHARGE ADVICE:

- RT feed as dietician advise
- To continue medications as per advise
- To continue physiotherapy
- Can take complete head bath with shampoo.
- Keep the wound clean and dry
- To continue oro-motor exercise, thermotactile stimulation & sour bolus strategy
- To do S. Sodium after 3 days & fludrocortisone & ADDNA follow by serum sodium once every week
- Thyroid profile after 1 month

IN CASE OF EMERGENCY CONTACT, NO: +91 - 44 - 4200 4200 (24*7)

FOR NEURO APPOINTMENTS PLEASE CONTACT: 7845936365

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